

Appendix — Technical Drawing Brief / Visual Architecture Reference

Version: v2 — updated 22 July 2026, 19:46 BST (interim label pending the versioning scheme in ProjectFiles.md), incorporating locked terminology and diagram corrections from the architecture-diagram review pass.

Status: DRAFT — supersedes v1 content in the sections marked below; v1 remains preserved as a separate file for provenance.

Purpose: This is a diagram-production reference for the Developer RFP / Intervention Test Build Specification. It is not itself the supplier RFP.

Use this page to create or check technical diagrams that explain how OTOS Continuity works.

What changed in v2: (1) terminology updated to **Partner Dedicated Node** throughout, replacing the earlier "Continuity Interface" / plain "partner interface" naming for the Layer-side connection point; (2) an explicit rule added against drawing partner-to-partner pass-through; (3) a new required diagram — **Diagram 2: Partner Interaction Close-up** — added to show per-partner interaction variety and volume. Everything else in v1 (the 3-layer/5-layer structure, the data model, the ADHD intervention sequence, the visual style, and the "what not to show" list) is unchanged and remains correct.

Title

OTOS Continuity™ Technical Architecture
Partner Interfaces, Continuity Layer and User Threads

Purpose

Create a proper technical diagram showing how OTOS Continuity works.

This is for developers, product architects, IG / DPO reviewers, governance reviewers and technical suppliers.

It must show the actual build model clearly:

- partner services keep their own routes and records;
- each partner has an authorised Interface / Login into OTOS;
- the partner Interface / Login connects to that partner's own **Partner Dedicated Node** on the Continuity Layer;
- the Continuity Layer contains tokenised user threads;
- partner actions create thread events;
- partner-approved prompts / check-ins can be sent from the Continuity Layer to the user;
- user responses / silence / opt-outs / attendance / drift signals are recorded back onto the thread;
- authorised partners see only agreed thread elements;
- evidence exports and dashboards are generated from thread events;
- OTOS does not create a clinical pathway, shared clinical record or EPR.

Required diagram structure

Use a layered architecture diagram (3 conceptual layers, shown across 5 visual bands as in v1).

Layer 1 — Existing services and service records

Place partner services across the top or left: CGL / CRS, ADHD Team, RCE / LAMP / Waiting-Well, Alcohol Liaison, and optional Phase 2 routes (GP, Mental Health, Mind, WorkWell, Therapy / VCSE).

Each service should have its own box: service record stays separate; normal service communication remains separate; clinical/service responsibility remains with partner.

Show grey lines from each service directly to the user labelled **"Existing service route / normal contact."** This must show that OTOS does not interfere with normal partner-user communication.

Locked rule (new in v2): each partner's grey "existing service route" line must run independently from that partner straight to the single central person and terminate there. **No grey line may pass through the person to reach another partner's box on the opposite side of the diagram** — even where two partners sit at the same height and a straight line would visually appear to connect them. Every partner needs its own visible endpoint at the person (an arrowhead, a stop, a junction marker) so six independent lines never read as three lines linking partners to each other. This applies equally to the gold Continuity Layer connections in Layer 2/3 below — no gold connection may pass through a Partner Dedicated Node or through the Layer to reach another partner.

Layer 2 — Partner Interface / Login → Partner Dedicated Node

Under or beside each partner service, show two connected elements, each partner's own pair:

1. **Authorised Interface / Login** — e.g. "CGL / CRS Interface / Login," "ADHD Team Interface / Login," "RCE Interface / Login," "Alcohol Liaison Checkpoint Interface / Login." Each interface can perform only agreed actions.
2. **Partner Dedicated Node** **RENAMED IN V2** — that partner's own registration point, continuity entry point and visibility point on the Layer. The compact label **Partner Node** may be used where space is tight.

Show action chips against the Interface / Login: view authorised thread elements; log touchpoint; create handoff receipt; trigger partner-approved prompt; update next-step status; review assigned drift / silence signal.

Important label: **Interface / Login = doorway for the partner; Partner Dedicated Node = that partner's own point of entry and visibility on the Continuity Layer itself.** A Partner Dedicated Node is not another service, not a shared clinical record, not another partner's interface, not a partner-to-partner connection, and not the endpoint of the partner's existing/direct service relationship (that stays on the grey line in Layer 1).

Layer 3 — Consent-led Continuity Layer

Draw a large gold layer / ring / platform labelled **"Consent-led Continuity Layer,"** visually layered above and around the grey Layer-1 infrastructure — never behind it, never merged with it.

Inside it show tokenised user threads. Each user thread contains: ConsentRecord, ParticipantToken, ContactMethod, CommunicationPreference, Touchpoints, HandoffReceipts, MessageEvents, SmsDeliveryReceipts, CheckInResponses, DriftSignals, HumanReviews, ReviewDecisions, EvidenceExport fields, AuditEvents.

Show partner actions flowing into this layer via each partner's own Partner Dedicated Node — never directly, never shared: "Partner action → own Partner Dedicated Node → Thread event recorded." Examples: CGL logs touchpoint → Touchpoint event; RCE logs attendance → Attendance event; ADHD Team updates next step → ADHD next-step status event; Alcohol Liaison logs checkpoint → Hospital checkpoint event.

Layer 4 — User interaction

Place the user / tokenised participant at the centre. Show: Continuity Layer → partner-approved SMS / prompt / check-in → User; and User → reply / silence / opt-out / attendance / missed handoff → Continuity Layer. Label: "Partner-approved instructions can be delivered through OTOS. User response or silence is recorded back onto the thread." Important: show that the user still has direct service routes outside OTOS (Layer 1's grey lines) — the Layer's channel and the existing-service channel are two separate lines to the same person, never one line doing both jobs.

Layer 5 — Outputs

On the right side, show outputs generated from thread events: role-based dashboard; human-review queue; open actions; thread activity; partner action list; SMS delivery / reply / silence; handoff receipts; drift / silence signals; evidence export; audit log. Label: "Outputs are generated from agreed thread events, not clinical records."

ADHD intervention detail

The diagram must make the ADHD intervention visible: suspected ADHD-driven addiction pattern recognised → consent-led thread opened → ADHD clinical-route / next-step status recorded where agreed → appointment booked → appointment attended → clinical team owns diagnosis/treatment/medication decision → recovery support, CBT, waiting-well and behavioural reinforcement continue → thread records touchpoints, replies, drift, handoffs and review actions.

Warning box, unchanged: OTOS does not diagnose ADHD; does not prescribe; does not manage medication; does not provide crisis care or safeguarding decisions; medication-status category only where clinically/governance approved.

Diagram 2 — Partner Interaction Close-up NEW IN V2

Added following Dean's direction (22 July 2026): partners will have multiple, varied daily interactions on the Thread, not just a single basic "attended: yes" touchpoint — this needs its own diagram, separate from the main architecture diagram, so the variety and volume don't clutter Diagrams 1.

Purpose

Show, for one partner at a time (e.g. RCE / Waiting-Well, or CGL / CRS), the realistic range and frequency of interactions that partner's own Partner Dedicated Node can send and receive on a single user's Continuity Thread — and make clear that even the smallest signal (a delivery receipt) is a legitimate, useful Thread event, not a lesser one.

What this diagram must show

- One partner (labelled), its Interface / Login, and its own Partner Dedicated Node — using the same Layer-2 elements as Diagram 1, not a new mechanism.

- A single user's Continuity Thread, shown wide enough to carry **several interaction types on the same day**, each individually labelled — e.g. partner-approved SMS check-in, 3-minute meditation prompt, quick mood check, relapse-prevention micro-quiz, attendance confirmation, delivery receipt only (no reply).
- A frequency band or timeline (e.g. a day-by-day or week-by-week strip) showing that a partner may send more than one touch per day where agreed — this is a **design and cost choice, not a technical ceiling**: the SMS Engine's chosen aggregator provider has far more capacity than any realistic cadence will use, so the diagram should not imply a hard system limit.
- A clear note that every partner's specific content set (exact wording, exact touchpoint names, exact cadence) is **illustrative only in this diagram** and subject to that partner's own agreement before launch — per Master 06 Phase 0 readiness ("SMS template set," "partner training pack").

Illustrative partner examples (not yet agreed — placeholders only)

Partner	Illustrative interaction types (TBD per partner)	Illustrative frequency (TBD)
RCE / Waiting-Well	SMS check-in, course/attendance prompt, 3-minute meditation prompt, quick mood check	Example: up to ~4 touches/day — not yet agreed
CGL / CRS	Keyworker touchpoint receipt, relapse-prevention quick quiz, recovery-plan reminder	Not yet agreed — CRS keyworkers currently contact roughly every 2 weeks; micro check-ins are intended to sit <i>between</i> those contacts, not replace them
ADHD Team	Next-step status update, appointment reminder	Lower frequency — clinical-route cadence, not behavioural-reinforcement cadence
Alcohol Liaison	Checkpoint-only, single event per presentation	Not a recurring cadence — checkpoint-only route

Every row above is a placeholder for discussion, not a locked spec. None of it should be treated as agreed content until the named partner has signed off their own set.

Why this matters (for the diagram's own explanatory text)

Per the Science of Sticking companion document: new behaviour consolidates through repetition with gaps in between, and the gap between formal sessions is exactly where a newly-forming pattern is most likely to be lost. A higher volume of small, partner-approved touches — down to a bare delivery receipt confirming the phone is reachable — is a legitimate continuity signal in its own right, not a lesser one, because it keeps the Thread active in the gap. This diagram exists to make that volume/variety visible to the developer, so the build does not assume one touchpoint per week is the ceiling.

Required footer

Use this exact footer on every page: *"Shared thread, not shared clinical record. Separate service records remain separate. Authorised partner views only."*

Internal build rule

Small technical note: *"Build the thread, not a pathway."* If shown publicly, phrase it as: *"OTOS wraps a consent-led Continuity Thread around the user journey."*

Visual style

Clean white background; gold/orange Continuity Layer; blue CGL/CRS; purple ADHD Team; green RCE/LAMP; red/pink Alcohol Liaison; grey for existing service routes; black footer lock; simple icons; arrows with labels; enough spacing. Must not be crowded — split across pages if necessary.

What not to show

- OTOS as a clinical pathway; all partners seeing all data; OTOS holding clinical notes; OTOS writing into EPR; OTOS diagnosing ADHD; OTOS prescribing; OTOS managing medication;
 - Alcohol Liaison as a Phase 1 onboarding route; GP / Mental Health / Mind / WorkWell / Therapy as active Phase 1 routes unless labelled optional / Phase 2; automated clinical triage;
 - **NEW IN V2** any partner's existing-service line or gold Layer connection passing through the central person to another partner's box;
 - **NEW IN V2** Diagram 2's illustrative partner content (quiz names, exact cadence) presented as agreed or final.
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Final output

1. one layman technical architecture diagram (Diagram 1);
 2. one developer data-flow diagram if needed;
 3. one Partner Interaction Close-up diagram (Diagram 2, new in v2), producible per-partner once that partner's content is agreed;
 4. a brief legend explaining each arrow;
 5. a list of all labels used;
 6. a short "what this diagram does not mean" box.
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Change log

Date / time	Change	By
9 Jul 2026 (original)	Appendix created — Technical Drawing Brief / Visual Architecture Reference, v1.	OTOS build process (pre-Cowork)
	Terminology updated to Partner Dedicated Node throughout; added explicit anti-pass-through rule; added new Diagram 2 — Partner Interaction Close-up spec. 3-layer/5-layer structure, data model, ADHD sequence, visual style and "what not to show" list	

22 Jul 2026, 19:46 BST	unchanged. Superseded file: <code>Appendix - Technical Drawing Brief Visual Architecture.pdf</code> (v1, preserved unchanged for provenance).	Claude, on Dean's direct instruction
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Appendix — Technical Drawing Brief / Visual Architecture Reference — v2, 22 July 2026, 19:46 BST. Companion to Master 08 — Developer RFP / Intervention Test Build Specification v1.1. v1 preserved separately for provenance; where the two disagree, this v2 file is current. Version-numbering convention to be confirmed against ProjectFiles.md once supplied — rename on request.